

Progress Notes

Signed Jun 23, 2026

Progress Notes by Amanda Allan Vandlac, MD at 06/23/26 1500

Assessment / Plan

UTI due to extended-spectrum beta lactamase (ESBL) producing Escherichia coli

Start 14 day course of macrobid

Then continue daily low dose ppx x 3 months

Continue to implement estrace cream 2 x a week

She does not want to take d mannose/cran supp d/t concerns over seizure activity

SHE HAS MULTIDRUG RESISTANT UTIS, REFERRING TO INFECTIOUS DISEASE FOR FURTHER ANTIBIOTIC MANAGEMENT AS WE HAVE COMPLETED AN ANATOMIC WORKUP THAT WAS NEGATIVE AND HAVE IMPLEMENTED ALL METHODS FOR PREVENTION

Dysuria

PYRDIUM prescribed. Parq held

1. Burning with urination

US Bladder Post Void Residual

2. UTI due to extended-spectrum beta lactamase (ESBL) producing Escherichia coli

* Prov Infectious Disease Consultants - AMB Referral

3. Dysuria

Orders Placed This Encounter

Procedures

- US Bladder Post Void Residual
- * Prov Infectious Disease Consultants - AMB Referral

No follow-ups on file.

Subjective

History of Present Illness:

Marie is a 63 y.o. female

Patient with history of chronic pain, COPD, DVT, SIADH, PTSD, epilepsy, left parietal occipital encephalomalacia following meningioma resection

Hx of recurrent multidrug resistant Ecoli UTIs

Here today d/t dysuria

Submitted a urine sample last week

Urine culture 6/18/26 - 100k ecoli (R to amp/sulbactam, cefazolin, ceftazidime, ceftriaxone, cipro, gent, levo, bactrim)

Cysto in 2026 neg for nidus for infection

CT 11/2025 - no nidus for infection

Hx of kidney stones: no

She has chronic midline abd pain not associated w voiding

She has recurrent sx of vaginitis - burning w urination with neg urine culture

Methods for UTI prevention:

- She was previously on cran supp and d mannose but had increased seizures with this so they stopped both
- At her last apt urine cx grew multidrug res ecoli - started on macrobid 100 mg bid x 14 days. Plan for daily low dose ppx x 3 months as well
- Estrace cream

Cr 0.51-12/5/2025

Urine cx Ecoli multidrug res 6/2026

Urine cx Ecoli multidrug res 1/22/26

Urine cx 100 K E. coli-11/1/2025

Urine culture E. coli/Proteus-9/22/2025

Urine culture Proteus-8/5/2025

Hx of OAB

- started on myrbetriq 50 mg daily at last apt

Pvr 0cc

No results found for: "PSA", "PSAFREE", "LABPSA", "PSADIAG", "PSAEX", "FRTOTPSA", "PSATOTAL", "PSAFR"

CT Head wo Contrast

Result Date: 6/9/2026

IMPRESSION: 1. No acute intracranial process evident. 2. Postsurgical changes stable.

Dictated by: Charles Croteau, DO on 6/9/2026 1:02 PM PDT Electronically signed by:

Charles Croteau, DO on 6/9/2026 1:05 PM PDT

XR Chest PA and Lateral

Result Date: 4/27/2026

IMPRESSION: 1. Multiple nondisplaced right 5-8 posterior lateral rib fractures further detailed on prior CT. No pneumothorax. 2. Low lung volumes with moderate perihilar and bibasilar atelectasis. Dictated by: Dennis Barbon, M.D. on 4/27/2026 1:01 PM PDT

XR Tibia Fibula Right 2 Vw

Result Date: 4/25/2026

IMPRESSION: Unremarkable right foreleg. Dictated by: Deborah L Arnold, M.D. on 4/25/2026 12:29 PM PDT Electronically signed by: Deborah L Arnold, M.D. on 4/25/2026 12:30 PM PDT

CT Angio Pulmonary w Contrast

Result Date: 4/24/2026

IMPRESSION: No pulmonary embolus visualized. Atelectasis is again identified throughout the lungs. Acute fractures of right ribs 5 through 8 at the posterior lateral aspect with adjacent pleural thickening and possible trace pleural effusion. Dictated by: Jenny H Wang, DO on 4/24/2026 6:49 PM PDT Electronically signed by: Jenny H Wang, DO on 4/24/2026 6:56 PM PDT

CT Lumbar Spine w Contrast

Result Date: 4/24/2026

IMPRESSION: 1. No acute fracture or dislocation evident. 2. Loss of height L1 similar compared to prior MRI, retropulsion of bone 4 mm similar compared to prior exam with mild central canal stenosis. Cortical irregularity ventral margin S1 vertebral body similar compared to prior exam. Mild concavity superior endplate of L4 similar compared to prior exam. Dictated by: Charles Croteau, DO on 4/24/2026 6:40 PM PDT Electronically signed by: Charles Croteau, DO on 4/24/2026 6:44 PM PDT

CT Thoracic Spine w Con Reconstruction

Result Date: 4/24/2026

IMPRESSION: 1. No acute fracture or dislocation evident within the thoracic spine. 2. Loss of height T6, T7, T8, and T11 vertebral bodies similar compared to prior exam and appears chronic. 3. Cortical lucencies posterior right rib 8. Acute or subacute fracture is not excluded. Correlate for point tenderness in the area. Dictated by: Charles Croteau, DO on 4/24/2026 6:34 PM PDT Electronically signed by: Charles Croteau, DO on 4/24/2026 6:40 PM PDT

CT Head wo Contrast

Result Date: 4/24/2026

IMPRESSION: 1. No intracranial hemorrhage or calvarial fracture evident. 2. Postsurgical changes stable. Dictated by: Charles Croteau, DO on 4/24/2026 6:31 PM PDT Electronically signed by: Charles Croteau, DO on 4/24/2026 6:34 PM PDT

Post-void residual 2/16/2026: 55ml

AUA Symptom Score 2/16/2026: / QOL:

Review of Systems: 12-point review of systems negative except as noted in HPI

Past Medical History: Marie has a past medical history of Anxiety, Arthritis, Asthma (HCC), Brain cancer (HCC) (1995), Chronic pain disorder, COPD (chronic obstructive pulmonary disease) (HCC), Deep vein thrombosis (HCC) (2020-2025), Depression, DVT, lower extremity, recurrent (HCC), Dyslipidemia, Encounter for blood transfusion, Headache, History of non-epileptiform seizure-like events (05/16/2019), resection of

meningioma, Hypothyroidism, Localization-related epilepsy (HCC), OSA (obstructive sleep apnea), Osteomyelitis of lumbar spine (HCC), SIADH (syndrome of inappropriate ADH production) (HCC), TBI (traumatic brain injury) (HCC), and Urinary incontinence.

Past Surgical History: Marie has a past surgical history that includes brain tumor removal; Hysterectomy; subtotal colectomy; Lumbar spine surgery; eye muscle surgery (Bilateral, 03/30/2021); Appendectomy; Tonsillectomy; Lumbar spine surgery (N/A, 11/14/2022); Bone Biopsy (2/1/2023); Bone Biopsy (2/3/2023); Lumbar spine surgery (N/A, 9/8/2023); laparotomy (Anterior, 9/8/2023); brain surgery; Colon surgery; Spine surgery; and other surgical history.

Social History: Marie reports that she quit smoking about 24 years ago. Her smoking use included cigarettes. She started smoking about 44 years ago. She has a 30 pack-year smoking history. She has been exposed to tobacco smoke. She has never used smokeless tobacco. She reports that she does not currently use alcohol. She reports that she does not currently use drugs after having used the following drugs: Amphetamines.

Family History: family history includes Alcohol abuse in her mother; Breast cancer (age of onset: 60) in her mother; Cancer in her mother; Liver disease in her mother; No known problems in her father.

Allergies

Allergies

Allergen	Reactions
• Vancomycin <i>shortness of breath February 2023</i>	Shortness Of Breath
• Lacosamide <i>See ER visit on 2025</i>	Hives
• Penicillins <i>Rash in her youth</i>	Rash

Current Medications

Current Outpatient Medications

Medication	Sig	Dispense	Refill
• acetaminophen (TYLENOL) 500 mg tablet	Take 2 tablets by mouth 3 times daily as needed for Pain.		
• albuterol 90 mcg/puff inhaler	Inhale 1 puff into the lungs every 4 hours as needed for Wheezing, Shortness of Breath or Increased Work of Breathing And as needed.	1 Inhaler	11
• apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet by mouth 2 times	60 tablet	0

daily. Indications:
DVT/PE
TREATMENT

• atorvaSTATin (LIPITOR) 10 mg tablet	Take 1 tablet by mouth daily at bedtime.		
• B Complex-C (SUPER B COMPLEX PO)	Take by mouth.		
• budesonide- glycopyrrolate-formoterol (BREZTRI AEROSPHERE) 160-9- 4.8 mcg/puff inhaler	Inhale 2 puffs into the lungs 2 times daily.	32.1 g	3
• divalproex (DEPAKOTE) 500 mg DR tablet	Take 2 tablets by mouth Daily AND 3 tablets daily at bedtime.	450 tablet	3
• doxycycline monohydrate (MONODOX) 100 mg capsule	Take 1 capsule by mouth 2 times daily.	180 capsule	3
• DULoxetine (CYMBALTA) 60 mg DR capsule	Take 1 capsule by mouth Daily.	30 capsule	0
• estradiol (ESTRACE) 0.01% vaginal cream	Place 2 g vaginally (see instruction) Take once a night for 2 wks then take twice a week.	100 g	5
• gabapentin (NEURONTIN) 400 mg capsule	Take 1 capsule by mouth 3 times daily.	90 capsule	2
• levETIRAcetam (KEPPRA) 500 mg tablet	TAKE 1 TABLET BY MOUTH 2 TIMES A DAY	180 tablet	3
• levothyroxine (SYNTHROID) 50 mcg tablet	Take 1 tablet by mouth every morning (before breakfast).	30 tablet	0
• magnesium hydroxide (MILK OF MAGNESIA) 400 mg/5 mL suspension	Take 30 mLs by mouth Daily as needed for Constipation (no BM in 3 days).		
• Multiple Vitamin (MULTIVITAMIN ADULT PO)	Take 1 tablet by mouth Daily .		
• nitrofurantoin (MACROBID) 100 mg capsule	Take 1 capsule by mouth 2 times daily for 7 days.	14 capsule	0

	Indications: UTI - Lower		
• nitrofurantoin (MACRODANTIN) 100 MG capsule	Take 1 capsule by mouth Daily. Indications: UTI - Lower	90 capsule	0
• oxyCODONE (ROXICODONE) 5 mg tablet	Take 1 tablet by mouth every 4 hours as needed for Pain Hold if sedated/drowsy.	20 tablet	0
• phenazopyridine (PYRIDIUM) 200 mg tablet	Take 1 tablet by mouth 3 times daily as needed for Pain.	20 tablet	0
• senna (SENOKOT) 8.6 mg tablet	Take 1 tablet by mouth 2 times daily as needed for Constipation.	60 tablet	0
• sodium chloride 1 g tablet	Take 2 tablets by mouth 2 times daily.	120 tablet	2
• tocopherol (VITAMIN E) 180 mg (400 units) capsule	Take 1 capsule by mouth Daily.		

No current facility-administered medications for this visit.

Objective

Vitals: BP 119/74 | Pulse 78 | Ht 1.651 m (5' 5") Comment: per patient | Wt 77.1 kg (170 lb) Comment: per patient | BMI 28.29 kg/m²

Physical Exam

UA Results of last 48 hours

**** No results found for the last 48 hours. ****

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